



GYNECOLOGICAL AND OBSTETRICAL EMERGENCIES

Effective: January 1, 2026

Replaces: January 1, 2025

1. Patient Care Goals

- 1.1. Recognize serious conditions associated with pregnancy and hypertension
- 1.2. Prevention of eclampsia-related seizures
- 1.3. Provide adequate treatment for eclampsia-related seizures
- 1.4. Recognize serious conditions associated with hemorrhage during pregnancy even when hemorrhage or pregnancy is not apparent (e.g. ectopic pregnancy, abruptio placenta, placenta previa)
- 1.5. Provide adequate resuscitation for hypovolemia

2. BLS Treatment

- 2.1. Routine Medical Care – Adult **(700-S04)**
 - 2.1.1. **Oxygen** – titrate as appropriate
- 2.2. Treat for signs and symptoms of shock as appropriate **(700-A10)**
- 2.3. Consider left lateral recumbent position if appropriate, supine position may cause hypotension
- 2.4. Monitor vital signs frequently

3. ALS Treatment

- 3.1. **Vascular Access (IV)**, TKO
- 3.2. If patient was treated for any condition in this protocol, transport to Labor and Delivery center as outlined in **Policy 602**, unless patient meets specialty care criteria

4. Vaginal Bleeding (Postpartum Hemorrhage)

- 4.1. Significant immediate postpartum hemorrhage is defined as the loss of greater than 1,000 ml of blood up to 24 hours post-delivery
 - 4.1.1. Delayed postpartum hemorrhage can occur up to 12-weeks after delivery
- 4.2. Perform firm uterine massage
- 4.3. If within 3 hours of delivery and bleeding continues after firm uterine massage, administer **Tranexamic Acid (TXA) 1,000 mg IV in 100 ml Normal Saline** at a rate of 100 gtts/min (1.6 gtts/sec) over 10 minutes

5. Spontaneous Abortion

- 5.1. If estimated gestational age of the fetus is 20 weeks or greater, attempt resuscitation **(700-P18)**
- 5.2. If fetus is non-viable, save and transport any tissue or fetal remains with the mother
- 5.3. Have patient place a sanitary napkin or bulky dressings over vaginal opening, **do not** pack the vaginal opening

6. Pre-eclampsia with Severe Features

- 6.1. Inclusion Criteria:
 - 6.1.1. Female patient, more than 20 weeks gestation and up to 6 weeks postpartum
 - 6.1.2. Severe hypertension with systolic blood pressure (SBP) $\geq$ (greater than or equal to) 160 mmHg or diastolic blood pressure (DBP) $\geq$ 110 mmHg OR
 - 6.1.3. SBP $\geq$ 140 mmHg or DBP $\geq$ 90 mmHg and any of the following symptoms:



- 6.1.3.1. Severe headache
- 6.1.3.2. Confusion/altered mental status
- 6.1.3.3. Blurred vision
- 6.1.3.4. Right upper quadrant or epigastric pain
- 6.2. Administer **Magnesium Sulfate 4 gm IV** in **100 ml Normal Saline**, at a rate of 50 gtt/min (0.8 gtt/sec) over 20 minutes if SBP > 100 mmHg
 - 6.2.1. If patient reports significant pain at site of magnesium administration, slow the drip rate until pain is controlled

7. Eclampsia

- 7.1. Inclusion Criteria:
 - 7.1.1. Female patient, more than 20 weeks gestation and up to 6 weeks postpartum and
 - 7.1.2. Actively seizing or postictal
- 7.2. If actively seizing, administer **Midazolam** according to Protocol (700-A02) AND
 - 7.2.1. Administer **Magnesium Sulfate 4 gm IV** in **100 ml Normal Saline**, at a rate of 50 gtt/min (0.8 gtt/sec) over 20 minutes if SBP > 100 mmHg
- 7.3. If postictal, administer **Magnesium Sulfate 4 gm IV** in **100 ml Normal Saline**, at a rate of 50 gtt/min (0.8 gtt/sec) over 20 minutes if SBP > 100 mmHg

8. Breech Delivery

- 8.1. Allow delivery to proceed passively until the baby's waist appears. Gently rotate the baby's body to a face down position and continue with delivery
- 8.2. If head does not readily deliver insert a gloved hand into the vagina to relieve pressure on the umbilical cord and create an air passage for the infant
- 8.3. Monitor vital signs and infant condition frequently

9. Prolapsed Cord

- 9.1. Place mother in the supine position with the head lower than the hips
- 9.2. Insert a gloved hand into the vagina and gently push the presenting part of the baby (head or shoulder) off of the umbilical cord. **Do Not** tug on the umbilical cord
- 9.3. Place fingers on each side of the baby's nose and mouth, split fingers into a "V" to create an opening. **Do Not** attempt to re-position the cord. **Do Not** remove your hand. Cover the exposed umbilical cord with saline soaked gauze.

10. Limb Presentation

- 10.1. Place mother in knee to chest position
- 10.2. Transport emergently to closest hospital (**Policy 602**)

11. Pertinent Assessment Findings

- 11.1. Vital signs to assess for signs of shock (e.g., tachycardia, hypotension)
- 11.2. Abdominal exam (e.g., distension, rigidity, guarding)
- 11.3. If pregnant, evaluate fundal height

12. Key Documentation Elements

- 12.1. Document full vital signs and physical exam findings
- 12.2. Primary or secondary impression
- 12.3. Postpartum hemorrhage or vaginal bleeding in pregnancy
 - 12.3.1. Primary Impression – Vaginal bleeding
 - 12.3.2. Secondary Impression – Pregnancy complication
- 12.4. Pre-eclampsia with severe features



- 12.4.1. Primary Impression – Hypertension
- 12.4.2. Secondary - Pregnancy complication
- 12.5. Eclampsia
 - 12.5.1. Primary Impression – Seizure - Active or Seizure – Post
 - 12.5.2. Secondary Impression – Pregnancy complication
- 12.6. Uncomplicated delivery
 - 12.6.1. Primary Impression – Childbirth – Mother
- 12.7. Complicated delivery (breech delivery, limb/cord presentation)
 - 12.7.1. Primary Impression – Childbirth – Mother
 - 12.7.2. Secondary Impression – Pregnancy complication
- 12.8. Active labor
 - 12.8.1. Primary Impression – Pregnancy/labor



13. Pre-eclampsia with Severe Features / Eclampsia Flowchart

